



THE AMERICAN
BOARD OF SURGERY

Setting a Standard for Competency-Based Board Admissibility using the American Board of Surgery's Entrustable Professional Activities

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Standard Setting

- Nothing to Disclose

American Board of Surgery EPAs

- Shift toward competency-based assessment (CBA)
- Represent 'everyday physician activities' or standardized expected behaviors
- Intended to provide timely feedback to trainees, based on discrete, directly observable behaviors
- For each EPA
 - Description of activity
 - Functions, by phase of care (nonoperative/preop, intraop, post-op)
 - In scope versus out of scope
 - Behaviorally-anchored entrustment scale
 - 1 – Limited Participation
 - 2 – Direct Supervision
 - 3 – Indirect Supervision
 - 4 – Practice Ready
 - Behavioral anchors provided by phase of care

GS EPA Timeline

2018-2020		2020-2023						2023-2029		Post 2029	
<u>Pilot</u>	<u>Development & Comms</u>							<u>Participation Phase</u>			
		2023-2024	2024-2025	2025-2026	2026-2027	2027-2028	2028-2029			<u>Performance Requirement Phase</u>	
		<i>Progressive Engagement</i>	<i>MA used for feedback to trainees</i>								
			<i>Summative Entrustment for CCC</i>								
			<i>Research & Data Coll.</i>			<i>Summative Entrustment Submitted for Exam</i>	<i>Performance Requirement Comms</i>			<i>Performance Req'd for Exam</i>	

Standard Setting

- Need a principled way of determining what level of performance across EPAs is needed to be admissible to exam process
- Standard setting
 - Common for multiple choice exams
 - Also used for performance assessments

Complexity in EPAs

- 18 General Surgery EPAs, which vary in usage/exposure
- Formative MA Ratings versus Summative Entrustment Ratings
- Phase-of-Care ratings versus Overall Entrustment ratings
 - Phase of Care Ratings
 - Analytic rating – Clear behavioral anchors for each entrustment level for each phase of care for each EPA
 - Differ by EPA, depending on what phases of care are included
 - Overall Entrustment Ratings
 - Holistic rating – Based on entrustment by phase of care and ‘other info’
 - Descriptions for each entrustment level still provide holistic descriptors that could be used to generate the holistic rating

Summative Entrustment Data



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For instructions on how to complete this worksheet, please see this document.

EPA Name

Phase

Summative Entrustment

Number of
Assessments*

Number of Faculty
Raters

**EVALUATE AND MANAGE OF A PATIENT WITH
ABDOMINAL WALL HERNIA**

EVALUATE AND MANAGE OF A PATIENT WITH ABDOMINAL WALL HERNIA	Nonoperative/Preoperative Phase			
EVALUATE AND MANAGE OF A PATIENT WITH ABDOMINAL WALL HERNIA	Intraoperative Phase			
EVALUATE AND MANAGE OF A PATIENT WITH ABDOMINAL WALL HERNIA	Postoperative Phase			
EVALUATE AND MANAGE OF A PATIENT WITH ABDOMINAL WALL HERNIA	OVERALL ENTRUSTMENT			



How Do We Do This?

Step 1: Guidance to Program

- Develop guidance to programs for generating summative entrustment ratings
 - ABS-driven recommendations, using CBBA committee for feedback, will iterate if necessary
 - E.g., how to aggregate, how many MAs, recency of data
- Timing: 2026-2027

Step 2: Identify Core v. Non-Core EPAs

- Prerequisite to setting a standard
- Data-driven method, via expert panel
 - Part 1 – Presentation of Data
 - Goal is to understand how frequently each EPA is assessed and what entrustment level toward the end of training
 - Part 2 – Panelist ratings (maybe)
 - For each EPA, have panelists provide ratings about expected levels of entrustment for minimally admissible candidates
 - Aggregate & discuss
 - Part 3 – Come to consensus on Core v Non-Core EPAs
- Timing: Winter/Spring 2027

Step 3: Develop Recommended Admissibility Requirement

- Standard Setting Panel
- Dominant Profile Judgement method (Plake, Hambleton, & Jaeger, 1997)
 - Describe process & examples
 - Round 1 Individual Policy Development & Discussion
 - Round 2 Individual Policy Development & Discussion
 - Discussion & Consensus Building
- Outcome: *Recommended* Admissibility Policy
 - Overall minimum level of admissibility across all EPAs
 - Which EPAs will require 'practice ready' entrustment?
 - Is 'indirect supervision' entrustment acceptable for non-core EPAs?
- Timing: Winter/Spring 2027

Step 4: Specialty Board Review & Approval

- Review recommended admissibility policy
 - Criteria
 - Required level of performance
- Evaluate impact data using summative data
- Approval
- Timing: Summer/Fall 2028
 - Not collecting summative entrustment data until 2028 exam applications

Communications & Timing

- Communications - Timely communication will be key
 - Core/Non-Core Communication likely first
 - Admissibility Requirement Communication – Targeting 2028-2029
- Implementation of Admissibility Requirement
 - Move to competency-based, rather than time-based, admissibility
 - Timing: Post-2029
- What then?
 - Research
 - Impact on training/certification
 - And?



Thank you!

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